

Global Situation Report

Middle East Escalation of Conflict

Situation report
No. 9

11 June 2026
Data as of 9 June



World Health
Organization

OVERVIEW

KEY AFFECTED COUNTRIES	Displaced	Injured	Deaths
Iran	3 200 000	33 806	3 375
Lebanon	1 049 328	11 072	3613
Iraq	N/A	300	109
Israel	N/A	7 834	26

All numbers are estimates and may differ from other sources due to data cutoff times. Estimates may be revised upward or downward over time.

ATTACKS ON HEALTH CARE	Attacks verified by WHO	Injured	Deaths
Iran	26	0	11
Lebanon	202	393	134
Israel	9	0	0

As reported in WHO's Surveillance System for Attacks on Health Care (SSA), where active. There may be delays in verification and changes upwards or downwards due to data consolidation.



ACUTE HEALTH THREATS

Despite ongoing ceasefires, the situation remains fragile. Renewed hostilities between Israel and Iran on 7–8 June resulted in additional casualties and disruptions to civilian infrastructure, while in Lebanon, continued instability, renewed strikes, evacuation notices and access constraints continue to affect civilians and disrupt health services. Acute health threats include trauma and mass casualty risks, communicable disease risks linked to displacement and overcrowded shelters, rising measles and leishmaniasis in Syria, measles transmission in Jordan, and persistent fuel, power and supply-chain constraints affecting continuity of essential health services.

WHO GLOBAL AND REGIONAL RESPONSE

WHO continues to leverage alternative logistics and procurement modalities to sustain essential health services amid persistent global constraints in health commodity and fuel supply chains. WHO's Dubai logistics hub remains operational and continues to manage emergency health supplies and air charters, while country offices are advancing procurement and delivery of trauma, intensive care, noncommunicable disease, mental health and backup power supplies.

Risk communication, community engagement, surveillance and preparedness support continue across priority countries:

- WHO has disseminated regional preparedness guidance and community messages;
- In Lebanon, supported radio-nuclear and chemical, biological, radiological, nuclear, and explosive (CBRN) preparedness in Lebanon, and trauma and hospitalization for conflict-affected non-Lebanese patients;
- In Iran, trained health workers on mental health and psychosocial support (MHPSS);
- In Syria, WHO and partners continue to strengthen the Early Warning, Alert and Response System (EWARS), risk communication and community engagement (RCCE) activities, outbreak preparedness, vaccination and awareness activities in coordination with ministries of health and partners.

For additional information on the global situation and response, please visit <https://www.who.int/emergencies/situations/middle-east-conflict>. For details on attacks on health care, please visit <https://extranet.who.int/ssa/index.aspx>.

COUNTRY HIGHLIGHTS

ISLAMIC REPUBLIC OF IRAN

Situation

According to national authorities, 15 injuries were reported during the latest escalation on 8 June, of whom 14 had been discharged from hospital. Flight operations have returned to normal following temporary suspensions affecting several airports, while internet access has largely been restored although connectivity remains limited in some areas. Potential environmental health implications from attacks in southern Iran remain under assessment. Shortages and high prices of NCD medicines and medical supplies continue to be reported.

WHO response

Implementation of WHO support to the Ministry of Health continues, including procurement of generators and uninterruptible power supply systems for health facilities, with delivery expected within two weeks, and NCD supplies expected by late June. WHO is also strengthening mental health and psychosocial services (MHPSS), with 250 health workers trained across 29 provinces, 87 peer-support sessions reaching approximately 4,700 health workers, and individual psychological support provided to around 300 health workers, while supporting the integration of MHPSS into primary health care services.

LEBANON

Situation

Despite the ongoing ceasefire in Lebanon, the security situation remains highly volatile, with multiple recent attacks and evacuation notices reported particularly affecting the south of the country. Since 2 March, 3526 deaths and 10 733 injuries have been reported by the ministry of health, including more than 1023 deaths and over 2062 injuries since the ceasefire on 17 April.

Attacks on health care continue to affect service delivery. WHO has recorded 202 attacks on health care since 2 March, including 32 attacks during the last three weeks alone. On 2 June 2026, Jabal Amel Hospital in Tyre sustained critical damage, including to the emergency and intensive care departments, but remained operational, while health infrastructure and access to care in southern Lebanon remain severely constrained.

Approximately 135,000 internally displaced people remain in collective shelters, with shelter capacity critically stretched in several governorates. A rapid health assessment conducted across 86 communities and 72 health facilities found a widening gap between increasing health needs and declining health system capacity. Access to health care remains constrained by insecurity, affordability barriers, medicine shortages and reduced availability of specialized services, while health facilities continue to face workforce shortages, infrastructure damage, stock-outs and reduced service capacity. Mental health needs are increasing, sexual and reproductive health services remain limited, raising particular concern for an estimated 13,500 displaced pregnant women in shelters, including approximately 1,500 expected deliveries within the next month, and access to essential medicines continues to be a major concern.

Flexible and rapid funding is urgently required to sustain emergency health operations and uninterrupted lifesaving services. Enhanced access is critical to restore outreach services, expand shelter and community-based surveillance, improve early detection and response to communicable disease threats, and replenish essential medicines and supplies.

WHO response

To date, WHO has distributed 120 trauma backpacks to ambulance and emergency medical service teams, as well as 20 trauma and emergency surgery kits and 15 ICU ventilators to priority hospitals. WHO continues to support trauma-related hospitalization costs for non-Lebanese patients affected by the conflict, with 122 patients supported to date, complementing government efforts to ensure coverage for all affected populations. WHO also continues to cover treatment costs for individuals with severe mental health needs, remaining the only organization providing support for such cases.

The Inter-Agency Flash Appeal for Lebanon has been extended for an additional three months and seeks additional US\$331 million to address urgent humanitarian needs, including US\$74.3 million for health targeting 1 million people.

For additional information on the situation and response in the Eastern Mediterranean region, please visit <https://www.emro.who.int/emergencies/middle-east-conflict/>.

Funding updates

WHO's US\$ 30.3 million emergency appeal to support health operations across five Middle Eastern countries remains critically underfunded, with only 13% (US\$ 4.1 million) received so far.